

Processing Location :

Rd #68, Jubilee Hills, Hyderabad- 500033  
Email: centrallab@luciddiagnostics.com

Client : HCL AVITAS PVT LTD

Patient Name : Mrs.VIDYA SRIKESH

Age / Gender : 60 Y(s) / Female

Phone : 9492063115

Ref.Dr. : SELF

Req.No : DSN102500337 - 2500879501

Report Status : Final

Sample Type : Serum

Registered On : 18-10-2025 09:04

Collected On : 18-10-2025 09:07

Received On : 18-10-2025 12:19

Reported On : 18-10-2025 13:33

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## DEPARTMENT OF CLINICAL BIOCHEMISTRY

**Blood Urea Nitrogen (BUN)**

| TEST NAME  | RESULT  | UNITS | BIOLOGICAL REFERENCE INTERVAL |
|------------|---------|-------|-------------------------------|
| Blood Urea | : 20.81 | mg/dL | 17-43                         |

Method : GLDH Kinetic

**Interpretation: -****Increased In**

Impaired kidney function, congestive heart failure, salt and water depletion due to vomiting, diarrhea urinary tract obstruction, hemorrhage into GI tract.

**Decreased In**

Malnutrition, pregnancy, infancy, acromegaly, low protein- High carbohydrate diet, impaired absorption, some patients of nephrotic syndrome.

**Blood Urea Nitrogen** : 9.7 mg/dL 7.0-18.0

Method : Calculated

**Reference**

In blood, Urea is usually reported as BUN and expressed in mg/dl. BUN mass units can be converted to urea mass units by multiplying by 2.14. Beckman kit literature.

Note :- Suggested Clinical Correlation \*

**\*\*End Of Report\*\*****Dr.Vijay Babu B,MBBS.MD**  
Consultant Biochemistry**Dr.Sowmya Gayatri .C**  
M.B.B.S MD  
Consultant Biochemistry**Dr.S.A.Nabi Ph.D**  
Sr.Biochemist

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## DEPARTMENT OF CLINICAL BIOCHEMISTRY

## Calcium, Serum

| TEST NAME      | RESULT | UNITS | BIOLOGICAL REFERENCE INTERVAL |
|----------------|--------|-------|-------------------------------|
| Calcium, Serum | : 9.7  | mg/dL | 8.8-10.6                      |

Method : Arsenazo III

## Interpretation

## Increased :

Increased levels may be seen in hyperparathyroidism, vitamin D intoxication, milk-alkali syndrome, thiazide diuretic use, and granulomatous diseases

## Decreased :

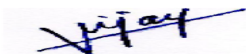
Hypoparathyroidism, chronic renal disease with uremia and phosphate retention, malabsorption of calcium and vitamin D, and insufficient calcium, phosphorus, and vitamin D intake.

Reference:

Beckman kit Insert.

Note :- Suggested Clinical Correlation \*

\*\*End Of Report\*\*

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## DEPARTMENT OF CLINICAL BIOCHEMISTRY

### Creatinine, Serum

| TEST NAME        | RESULT | UNITS | BIOLOGICAL REFERENCE INTERVAL |
|------------------|--------|-------|-------------------------------|
| Creatinine,Serum | : 0.72 | mg/dL | 0.6-1.2                       |

Method : Modified Jaffe Kinetic

**Reference** :Tietz fundamentals of clinical chemistry, 7 th edition.

#### Interpretation:

##### Increased In

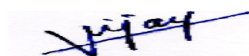
- \* Diet: ingestion of creatinine (roast meat)
- \* Muscle disease: gigantism, acromegaly
- \* Prerenal azotemia
- \* Impaired kidney function
- \* An increase in serum creatinine creatinine occurs in 10.20% of patients taking aminoglycosides and  $\leq 20\%$  of patients taking penicillins (especially methicillin)

##### Decreased In

- \* Creatinine secretion is inhibited by certain drugs (e.g., cimetidine, trimethoprim).
- \* Proxy for reduced skeletal muscle mass.

Note :- Suggested Clinical Correlation \*

**\*\*End Of Report\*\***



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Reported On : 18-10-2025 14:02

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## DEPARTMENT OF SEROLOGY

## High Sensitivity - CRP (HS-CRP)

| TEST NAME | RESULT | UNITS | BIOLOGICAL REFERENCE INTERVAL                                                  |
|-----------|--------|-------|--------------------------------------------------------------------------------|
| hs-CRP    | : 0.04 | mg/dL | Low risk : less than 0.1<br>Average risk : 0.1 to 0.3<br>High risk : above 0.3 |

Method : Immunoturbidimetry

## Reference :

The American Heart Association and U.S. Centers for Disease Control and Prevention.

## Note:

In addition to hs-CRP, other additional risk factors to be considered for cardiovascular diseases (CVD)

Note :- Suggested Clinical Correlation \*

\*\*End Of Report\*\*

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Consultant Microbiologist  
Dr.Sara Shireen MBBS MD  
Consultant Microbiologist

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## DEPARTMENT OF CLINICAL BIOCHEMISTRY

## Phosphorous, Serum

| TEST NAME          | RESULT | UNITS | BIOLOGICAL REFERENCE INTERVAL |
|--------------------|--------|-------|-------------------------------|
| Phosphorous, Serum | : 4.5  | mg/dL | 2.5 -4.5                      |

Method : Phosphomolybdate

Reference: Beckman Kit Insert.

Note :- Suggested Clinical Correlation \*

\*\*End Of Report\*\*

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DEPARTMENT OF CLINICAL BIOCHEMISTRY

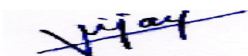
**Transferrin Saturation**

| TEST NAME                                                                       | RESULT   | UNITS | BIOLOGICAL REFERENCE INTERVAL |
|---------------------------------------------------------------------------------|----------|-------|-------------------------------|
| <b>Iron</b><br><i>Method : TPTZ</i>                                             | : 85.43  | µg/dL | 60-180                        |
| <b>Total Iron Binding Capacity (TIBC)</b><br><i>Method : Nitroso-PSAP</i>       | : 362.99 | µg/dL | 255-450                       |
| <b>Unsaturated Iron Binding Capacity (UIBC)</b><br><i>Method : Nitroso-PSAP</i> | : 277.56 | µg/dL | 155-300                       |
| <b>Transferrin Saturation</b><br><i>Method : Calculation</i>                    | : 23.54  | %     | 20-50                         |

**Reference:** Beckman kit Insert.

Note :- Suggested Clinical Correlation \*

**\*\*End Of Report\*\***



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DEPARTMENT OF CLINICAL BIOCHEMISTRY

Uric Acid, Serum

| TEST NAME | RESULT | UNITS | BIOLOGICAL REFERENCE INTERVAL |
|-----------|--------|-------|-------------------------------|
| Uric Acid | : 4.68 | mg/dL | 2.6-6.0                       |

Method : Uricase PAP(Phenyl AminoPhenazone)

Interpretation

Increased :

renal failure, gout, leukemia, polycythemia, and hemolytic anemia. decreased renal clearance due to drugs like thiazides

and furosemide, the use of drugs like barbiturates, some patients with alcoholism, high protein weight-reduction diet.

Decreased :

Use of uricosuric drugs like probenecid, Allopurinol, cortisone, estrogens, Fanconi syndrome, etc.

Note :- Suggested Clinical Correlation \*

\*\*End Of Report\*\*

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Report Status : Final

Sample Type : Serum

Registered On : 18-10-2025 09:04

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Received On : 18-10-2025 12:19

Reported On : 18-10-2025 15:53

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DEPARTMENT OF CLINICAL BIOCHEMISTRY

**Vitamin B12 (Cyanocobalamin),Serum**

| TEST NAME                           | RESULT | UNITS | BIOLOGICAL REFERENCE INTERVAL |
|-------------------------------------|--------|-------|-------------------------------|
| Vitamin B12 (Cyanocobalamin), Serum | : 877  | pg/mL | 222-1439                      |

Method : CLIA

**Interpretation:**

1.Vitamin B12 deficiency occurs in megaloblastic anaemia,malabsorption,dietary deficiency(vegeterians),congenital disorders,pregnancy etc.

2.An associated increase in homocysteine is an independent marker for cardiovascular disease and deep vein thrombosis.

3.Holotranscobalamin is a more sensitive marker of Vitamin B12 deficiency.

4.Elevated Vitamin B12 is associated with use of oral contraceptives,multivitamins ,Chronic renal failure,liver disease,increased levels of transcobalamin,acute and chronic myelogeneous leukemia etc.

**Note:** disagreement of vitamin b12 values can be noticed between laboratories using assays of different manufacturers.

**Reference:**Beckman caliper study of pediatric Reference values.Beckman kit insert,Teitz Fundamentals of clinical chemistry.

Note :- Suggested Clinical Correlation \*

**\*\*End Of Report\*\***

*Vijay*

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## DEPARTMENT OF CLINICAL BIOCHEMISTRY

### Lipid Profile

| TEST NAME                                                                                  | RESULT   | UNITS | BIOLOGICAL REFERENCE INTERVAL                                                                                 |
|--------------------------------------------------------------------------------------------|----------|-------|---------------------------------------------------------------------------------------------------------------|
| <b>Total Cholesterol</b><br><i>Method : CHOD-POD</i>                                       | : 181.7  | mg/dL | Desirable: < 200<br>Borderline: 200 – 239<br>High: >= 240                                                     |
| <b>HDL Cholesterol</b><br><i>Method : Enzymatic Immunoinhibition</i>                       | : 70.7   | mg/dL | Major risk factor for heart disease: <40<br>Negative risk factor for heart disease: >60                       |
| <b>LDL Cholesterol</b><br><i>Method : Calculation</i>                                      | : 99.79  | mg/dL | Optimal : < 100<br>Near Optimal : 100–129<br>Borderline High : 130-159<br>High : 160–189<br>Very High : > 190 |
| <b>VLDL</b><br><i>Method : Calculated</i>                                                  | : 11.21  | mg/dL | 6-38                                                                                                          |
| <b>Triglycerides</b><br><i>Method : Glycerol Phosphate Oxidase (GPO), Peroxidase (POD)</i> | : 56.03  | mg/dL | Normal: < 150<br>Borderline High: 150 - 199<br>High: 200 - 499<br>Very High: >= 500                           |
| <b>CHOL / HDL Ratio</b><br><i>Method : Calculation</i>                                     | : 2.57   |       | 3.5-5.0                                                                                                       |
| <b>Non - HDL Cholesterol</b><br><i>Method : Calculated</i>                                 | : 111.00 | mg/dL | <130                                                                                                          |
| <b>LDL/HDL Ratio</b><br><i>Method : Calculated</i>                                         | : 1.41   | mg/dL | 1.3-3.5                                                                                                       |

**Reference:**The National Cholesterol Education Program Adult Treatment Panel III (NCEP-ATP III) Guidelines.

Note :- Suggested Clinical Correlation \*

\*\*End Of Report\*\*

  
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Print Date : 21-10-2025 10:11

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Reported On : 18-10-2025 15:59

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## DEPARTMENT OF CLINICAL BIOCHEMISTRY

### Thyroid Profile (T3,T4,TSH)

| TEST NAME                               | RESULT  | UNITS  | BIOLOGICAL REFERENCE INTERVAL                                                                                                     |
|-----------------------------------------|---------|--------|-----------------------------------------------------------------------------------------------------------------------------------|
| <b>Total T3</b><br><i>Method : CLIA</i> | : 1.06  | ng/mL  | 0.87-1.78                                                                                                                         |
| <b>Total T4</b><br><i>Method : CLIA</i> | : 10.00 | µg/dL  | 4.82-15.65                                                                                                                        |
| <b>TSH</b><br><i>Method : CLIA</i>      | : 1.81  | µIU/mL | <b>Children:</b><br>Birth-4d:1.0-39.0<br>2-20wk:1.7-9.1<br>21wk-20y:0.7-6.4<br><b>Adults:</b><br>21-54y:0.4-4.2<br>55-87y:0.5-8.9 |

#### Interpretation :

1.T3 &T4 values may be altered due to changes in serum proteins,pregnancy,drugs,nephrosis etc.In such cases Free T3 and Free T4 may give more appropriate thyroid status.T3 levels fluctuate rapidly to stress and non thyroid illness.

2.TSH values may be transiently altered in fever,severe infections,liver disease,renal and heart failure,severe burns,trauma and surgery.

3.Drugs that decrease TSH values include L-DOPA,Glucocorticoids,Heparin.Drugs that increase TSH include-Iodine,Lithium,Amiodarone.

4.TSH exhibits diurnal rythm,peaks at 2.00- 4.00AM and has lowest level at 5.00-6.00PM

**Reference:** Beckman caliper study of pediatric Reference values.Beckman kit insert,Teitz Fundamentals of clinical chemistry.

Note :- Suggested Clinical Correlation \*

\*\*End Of Report\*\*



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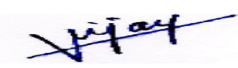
## DEPARTMENT OF CLINICAL BIOCHEMISTRY

### Liver Function Test (LFT)

| TEST NAME                                                             | RESULT  | UNITS | BIOLOGICAL REFERENCE INTERVAL |
|-----------------------------------------------------------------------|---------|-------|-------------------------------|
| <b>Total Bilirubin</b><br><i>Method : DPD</i>                         | : 0.60  | mg/dL | 0.3-1.2                       |
| <b>Direct Bilirubin</b><br><i>Method : DPD</i>                        | : 0.13  | mg/dL | <0.2                          |
| <b>Indirect Bilirubin</b><br><i>Method : Calculated</i>               | : 0.47  | mg/dL | 0.0-0.9                       |
| <b>SGPT / ALT</b><br><i>Method : IFCC without Pyridoxal Phosphate</i> | : 15.30 | U/L   | 0-35                          |
| <b>AST/SGOT</b><br><i>Method : IFCC without Pyridoxal Phosphate</i>   | : 25.45 | U/L   | 0-35                          |
| <b>Alkaline Phosphatase</b><br><i>Method : IFCC AMP-Buffer</i>        | : 79.16 | IU/L  | 30-120                        |
| <b>Total Protein (TP)</b><br><i>Method : Biuret</i>                   | : 7.43  | g/dL  | 6.6-8.3                       |
| <b>Albumin</b><br><i>Method : Bromocresol Green(BCG)</i>              | : 4.07  | g/dL  | 3.5-5.2                       |
| <b>Globulin</b><br><i>Method : Calculation</i>                        | : 3.4   | g/dL  | 1.8-3.8                       |
| <b>A/G Ratio</b><br><i>Method : Calculated</i>                        | : 1.20  |       | 0.9-1.8                       |

Note :- Suggested Clinical Correlation \*

\*\*End Of Report\*\*

  
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|               |                             |               |                    |                                                                                     |
|---------------|-----------------------------|---------------|--------------------|-------------------------------------------------------------------------------------|
| Client        | : HCL AVITAS PVT LTD        | Sample Type   | : WHOLE BLOOD EDTA | Scan to Validate                                                                    |
| Patient Name  | : Mrs.VIDYA SRIKESH         | Registered On | : 18-10-2025 09:04 |  |
| Age / Gender  | : 60 Y(s) / Female          | Collected On  | : 18-10-2025 09:07 |                                                                                     |
| Phone         | : 9492063115                | Received On   | : 18-10-2025 12:19 |                                                                                     |
| Ref.Dr.       | : SELF                      | Reported On   | : 18-10-2025 17:05 |                                                                                     |
| Req.No        | : DSN102500337 - 2500879500 |               |                    |                                                                                     |
| Report Status | : Final                     |               |                    |                                                                                     |

DEPARTMENT OF CLINICAL BIOCHEMISTRY

**Glycosylated Hb (HbA1C)**

| TEST NAME                                              | RESULT   | UNITS | BIOLOGICAL REFERENCE INTERVAL                       |
|--------------------------------------------------------|----------|-------|-----------------------------------------------------|
| <b>Glycosylated Hb (HbA1C)</b>                         | : 5.6    | %     | Normal:<5.7<br>Prediabetes:5.7-6.4<br>Diabetes:>6.5 |
| <i>Method : High-performance liquid chromatography</i> |          |       |                                                     |
| <b>Estimated Average Glucose</b>                       | : 114.02 | mg/dL |                                                     |
| <i>Method : Calculated</i>                             |          |       |                                                     |

**Use:**

HbA1C reflects the mean blood glucose concentration over the previous 3-4 months.

**Interpretation:**

**Criteria for diagnosis of Diabetes:**

HbA1C  $\geq$  6.5% using method that is NGSP certified & standardised to DCCT Assay.

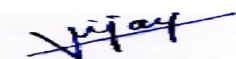
**Note:**

Low HbA1C values(<4%) in a individual are often associated with systemic inflammatory diseases,chronic anemia,chronic renal failure and liver diseases.

**Reference:**American Diabetes Association guidelines 2022.

Note :- Suggested Clinical Correlation \*

**\*\*End Of Report\*\***



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Req.No : DSN102500337 - 2500880853

Report Status : Final

Sample Type : Plasma - PP

Registered On : 18-10-2025 09:04

Collected On : 18-10-2025 12:23

Received On : 18-10-2025 14:11

Reported On : 18-10-2025 17:05

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DEPARTMENT OF CLINICAL BIOCHEMISTRY

**Glucose, Post Lunch (PLBS)**

| TEST NAME                 | RESULT | UNITS | BIOLOGICAL REFERENCE INTERVAL |
|---------------------------|--------|-------|-------------------------------|
| Post Lunch Plasma Glucose | : 98   | mg/dL | 100-140                       |

Method : Hexokinase

**Remarks:**

Postprandial hypoglycemia may be observed in persons taking certain diabetes medications, antibiotics and alcoholic beverages. Additionally intestinal disorders, endocrine disorders, insulin surge, nature of diet, reduced food intake than usual and stress are among several other factors. Incretin effect: Incretins released by the nueroendocrine cells of the GI tract can cause an insulin surge that can lead to lowering of the blood glucose level in the post prandial state, some times to levels below the fasting level. Please correlate clinically.

**Reference:**American Diabetes association guidelines 2021.

Note :- Suggested Clinical Correlation \*

**\*\*End Of Report\*\***

*Vijay*

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M.B.B.S MD  
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**Dr.S.A.Nabi Ph.D**  
Sr.Biochemist

Processing Location :

Rd #68, Jubilee Hills, Hyderabad- 500033  
Email: centrallab@luciddiagnostics.com

Client : HCL AVITAS PVT LTD

Patient Name : Mrs.VIDYA SRIKESH

Age / Gender : 60 Y(s) / Female

Phone : 9492063115

Ref.Dr. : SELF

Req.No : DSN102500337 - 2500879503

Report Status : Final

Sample Type : Urine

Registered On : 18-10-2025 09:04

Collected On : 18-10-2025 09:07

Received On : 18-10-2025 12:19

Reported On : 18-10-2025 16:49

Scan to Validate



## DEPARTMENT OF CLINICAL PATHOLOGY

### Complete Urine Examination (CUE)

| TEST NAME | RESULT | UNITS | BIOLOGICAL REFERENCE INTERVAL |
|-----------|--------|-------|-------------------------------|
|-----------|--------|-------|-------------------------------|

#### PHYSICAL EXAMINATION:

**Colour** : Pale Yellow STRAW TO YELLOW

**Appearance** : Clear

#### CHEMICAL EXAMINATION

**Reaction (pH)** : Acidic 6.0 5.0-8.0

Method : Methyl Red & Bromothymol Blue

**Specific Gravity** : 1.015 1.000-1.030

Method : pKA Change

**Protein / Albumin** : Negative NEGATIVE

Method : Tetrabromophenol blue

**Glucose / Sugar** : Negative NEGATIVE

Method : Glucose oxidase/Peroxidase

**Blood** : Positive(+) NEGATIVE

Method : Peroxidase

**Ketones** : Negative NEGATIVE

Method : Sodium Nitroprusside

**Bilirubin** : Negative NEGATIVE

Method : Dichloroanilinediazonium

**Leucocytes** : Negative NEGATIVE

Method : 3 hydroxy5 phenylpyrrole + diazonium

**Nitrites** : Negative NEGATIVE

Method : Diazonium + 1,2,3,4 tetrahydrobenzo (h) quinolin 3-ol

**Urobilinogen** : 0.2 mg/dL 0.2-1.0

Method : Dim ethyl aminobenzaldehyde

#### MICROSCOPIC EXAMINATION:

**Pus Cells** : 3-4 /HPF 0-5

**Epithelial Cells** : 2-3 /HPF 0-5

**RBC** : 8-10 /HPF Absent

**Casts** : Absent Absent

**Crystals / Amorphous deposit** : Absent Absent

**Others** : Nil Nil

**Method:** Reflectance Photometry, Automated Urine Analyzer/Microscopy.

Print Date : 21-10-2025 10:11

Page 14 of 21

**Regd. Office:**

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Ph: +91 40 4419 4444, Toll Free No. : 1800 123 4577

Processing Location :

Rd #68, Jubilee Hills, Hyderabad- 500033  
Email: centrallab@luciddiagnostics.com

Client : HCL AVITAS PVT LTD

Patient Name : **Mrs.VIDYA SRIKESH**

Age / Gender : **60 Y(s) / Female**

Phone : 9492063115

Ref.Dr. : SELF

Req.No : **DSN102500337** -

**Report Status : Final**

Sample Type : Urine

Registered On : 18-10-2025 09:04

Collected On : 18-10-2025 09:07

Received On : 18-10-2025 12:19

Reported On : 18-10-2025 16:49

Scan to Validate



**Reference:**

Siemens Multistix 10 SG Kit insert. Textbook of Medical Laboratory Technology B. Godkar, 3rd Edition.

Note :- Suggested Clinical Correlation \*

**\*\*End Of Report\*\***

**Dr.A.Ramya Priyadarshini**  
Consultant Pathologist



**Dr.K Kiran**  
Consultant Pathologist

**Dr.Gaurav Rastogi**  
MD, Pathology  
Senior Consultant

**Dr.Sushmitha Reddy A**  
Consultant Pathologist



Processing Location :

Rd #68, Jubilee Hills, Hyderabad- 500033  
Email: centrallab@luciddiagnostics.com

|               |                             |               |                                     |
|---------------|-----------------------------|---------------|-------------------------------------|
| Client        | : HCL AVITAS PVT LTD        | Sample Type   | : WHOLE BLOOD EDTA Scan to Validate |
| Patient Name  | : Mrs.VIDYA SRIKESH         | Registered On | : 18-10-2025 09:04                  |
| Age / Gender  | : 60 Y(s) / Female          | Collected On  | : 18-10-2025 09:07                  |
| Phone         | : 9492063115                | Received On   | : 18-10-2025 12:19                  |
| Ref.Dr.       | : SELF                      | Reported On   | : 18-10-2025 15:06                  |
| Req.No        | : DSN102500337 - 2500879500 |               |                                     |
| Report Status | : Final                     |               |                                     |



## DEPARTMENT OF HAEMATOLOGY

### Complete Blood Picture (CBP)

| TEST NAME                                                                | RESULT | UNITS         | BIOLOGICAL REFERENCE INTERVAL |
|--------------------------------------------------------------------------|--------|---------------|-------------------------------|
| <b>Hemoglobin</b><br><i>Method : Non-Cyanide Photometric Measurement</i> | : 12.1 | g/dL          | 12.0-15.0                     |
| <b>HCT/Haematocrit</b><br><i>Method : Calculated</i>                     | : 35.4 | %             | 36.0-46.0                     |
| <b>RBC Count</b><br><i>Method : Electrical Impedance</i>                 | : 3.94 | millions/cumm | 3.8-4.8                       |
| <b>MCV</b><br><i>Method : Calculated</i>                                 | : 89.7 | fL            | 83.0-101.0                    |
| <b>MCH</b><br><i>Method : Calculated</i>                                 | : 30.7 | pg            | 27.0- 32.0                    |
| <b>MCHC</b><br><i>Method : Calculated</i>                                | : 34.3 | g/dL          | 31.5-34.5                     |
| <b>RDW-CV</b><br><i>Method : Calculated</i>                              | : 13.4 | %             | 11.6-14.0                     |
| <b>RDW-SD</b><br><i>Method : Calculated</i>                              | : 42.9 | fL            | 39-46                         |
| <b>MPV</b><br><i>Method : Calculated</i>                                 | : 7.2  | fL            | 7.40-10.40                    |
| <b>Platelet Count</b><br><i>Method : Electrical Impedance</i>            | : 3.20 | lakhs/cmm     | 1.50-4.10                     |
| <b>Total WBC Count</b><br><i>Method : Electrical Impedance</i>           | : 4600 | Cells/cmm     | 4000-11000                    |

### Differential Count

#### (Method:VCS/Leishman Stain/Microscopy)

|                    |      |   |       |
|--------------------|------|---|-------|
| <b>Neutrophils</b> | : 47 | % | 40-80 |
| <b>Lymphocytes</b> | : 43 | % | 20-40 |
| <b>Monocytes</b>   | : 08 | % | 2-10  |
| <b>Eosinophils</b> | : 02 | % | 1-6   |
| <b>Basophils</b>   | : 00 | % | 0-1   |

### PERIPHERAL BLOOD PICTURE

|                  |                           |
|------------------|---------------------------|
| <b>RBC</b>       | : Normocytic Normochromic |
| <b>WBC</b>       | : Relative Lymphocytosis  |
| <b>Platelets</b> | : Adequate                |

Print Date : 21-10-2025 10:11

Page 16 of 21

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Ph: +91 40 4419 4444, Toll Free No. : 1800 123 4577

Processing Location :

Rd #68, Jubilee Hills, Hyderabad- 500033  
Email: centrallab@luciddiagnostics.com

Client : HCL AVITAS PVT LTD

Patient Name : Mrs. VIDYA SRIKESH

Age / Gender : 60 Y(s) / Female

Phone : 9492063115

Ref.Dr. : SELF

Req.No : DSN102500337 -

Report Status : Final

Sample Type : WHOLE BLOOD EDTA Scan to Validate

Registered On : 18-10-2025 09:04

Collected On : 18-10-2025 09:07

Received On : 18-10-2025 12:19

Reported On : 18-10-2025 15:06



**Reference :** Dacie and Lewis Practical Hematology, 12th Edition

**Note :** These results are generated by a fully automated hematology analyzer and the differential count is done on a peripheral smear.

**Method:**

Fully automated haematology analyzer (Beckman Coulter DXH900) (Photometric Measurement, Electrical Impedance, VCS Technology, Leishman's Stain and Microscopy)

Note :- Suggested Clinical Correlation \*

**\*\*End Of Report\*\***

**Dr.Gaurav Rastogi MD, Pathology**  
Senior Consultant

**Dr.Sushmitha Reddy A**  
Consultant Pathologist

**Dr.K Kiran**  
Consultant Pathologist



**Dr.A.Ramya Priyadarshini**  
Consultant Pathologist

Print Date : 21-10-2025 10:11

**Regd. Office:**

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Processing Location :

Rd #68, Jubilee Hills, Hyderabad- 500033  
Email: centrallab@luciddiagnostics.com

|               |                             |               |                    |                                                                                     |
|---------------|-----------------------------|---------------|--------------------|-------------------------------------------------------------------------------------|
| Client        | : HCL AVITAS PVT LTD        | Sample Type   | : WHOLE BLOOD EDTA | Scan to Validate                                                                    |
| Patient Name  | : Mrs.VIDYA SRIKESH         | Registered On | : 18-10-2025 09:04 |  |
| Age / Gender  | : 60 Y(s) / Female          | Collected On  | : 18-10-2025 09:07 |                                                                                     |
| Phone         | : 9492063115                | Received On   | : 18-10-2025 12:19 |                                                                                     |
| Ref.Dr.       | : SELF                      | Reported On   | : 18-10-2025 14:54 |                                                                                     |
| Req.No        | : DSN102500337 - 2500879500 |               |                    |                                                                                     |
| Report Status | : Final                     |               |                    |                                                                                     |

## DEPARTMENT OF HAEMATOLOGY

### Erythrocyte Sedimentation Rate (ESR)

| TEST NAME      | RESULT | UNITS | BIOLOGICAL REFERENCE INTERVAL                      |
|----------------|--------|-------|----------------------------------------------------|
| E.S.R 1st HOUR | : 14   | mm/hr | Male: 0 – 10 mm/1st hr<br>Female: 0 – 15 mm/1st hr |

Method : Modified Westergren

#### Interpretation:

Erythrocyte sedimentation rate (ESR) is a useful but nonspecific marker of underlying inflammation. ESR is elevated in: Rheumatoid arthritis, chronic infection, collagen disease, polyclonal hyperglobulinemia and hyperfibrinogenemia Temporal arteritis, septic arthritis, pelvic inflammatory disease, and appendicitis, Osteomyelitis. Neoplastic disease- Myeloma or Macroglobulinemia, Prostate cancer, Hodgkin's disease, Renal cell carcinoma. Stroke, coronary artery disease Pregnancy - increase at the 10th to the 12th week, and returns to normal about 1 month postpartum. ESR is decreased in: polycythemia, hyperviscosity, sickle cell anemia, leukemia, low plasma protein (liver, kidney disease) and congestive heart failure.

Reference: Henry's Clinical Diagnosis and Management by Laboratory Methods, 22nd edition, page-521.

Note :- Suggested Clinical Correlation \*

**\*\*End Of Report\*\***

**Dr.Gaurav Rastogi MD,Pathology**  
Senior Consultant

**Dr.Sushmitha Reddy A**  
Consultant Pathologist

**Dr.K Kiran**  
Consultant Pathologist

  
**Dr.A.Ramya Priyadarshini**  
Consultant Pathologist

Processing Location :

Rd #68, Jubilee Hills, Hyderabad- 500033  
Email: centrallab@luciddiagnostics.com

Client : HCL AVITAS PVT LTD

Patient Name : Mrs.VIDYA SRIKESH

Age / Gender : 60 Y(s) / Female

Phone : 9492063115

Ref.Dr. : SELF

Req.No : DSN102500337 - 2500879501

Report Status : Final

Sample Type : Serum

Registered On : 18-10-2025 09:04

Collected On : 18-10-2025 09:07

Received On : 18-10-2025 12:19

Reported On : 18-10-2025 15:14

Scan to Validate



## DEPARTMENT OF CLINICAL BIOCHEMISTRY

**25 Hydroxy Vitamin D****25 Hydroxy Vitamin D.** : 33.08

ng/mL

Method : CLIA

Deficiency : &lt; 20

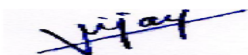
Insufficiency: 20 - 30

Sufficiency : 30 - 100

Upper limit of safety : &gt; 100

Note :- Suggested Clinical Correlation \*

\*\*End Of Report\*\*

**Dr.Vijay Babu B,MBBS.MD**  
Consultant Biochemistry**Dr.Sowmya Gayatri .C**  
M.B.B.S MD  
Consultant Biochemistry**Dr.S.A.Nabi Ph.D**  
Sr.Biochemist

Processing Location :

Rd #68, Jubilee Hills, Hyderabad- 500033  
Email: centrallab@luciddiagnostics.com

Client : HCL AVITAS PVT LTD

Patient Name : Mrs.VIDYA SRIKESH

Age / Gender : 60 Y(s) / Female

Phone : 9492063115

Ref.Dr. : SELF

Req.No : DSN102500337 - 2500879501

Report Status : Final

Sample Type : Serum

Registered On : 18-10-2025 09:04

Collected On : 18-10-2025 09:07

Received On : 18-10-2025 12:19

Reported On : 18-10-2025 13:33

Scan to Validate



## DEPARTMENT OF CLINICAL BIOCHEMISTRY

## Iron with TIBC

| TEST NAME                          | RESULT   | UNITS | BIOLOGICAL REFERENCE INTERVAL |
|------------------------------------|----------|-------|-------------------------------|
| Iron                               | : 85.43  | µg/dL | 60-180                        |
| Method : TPTZ                      |          |       |                               |
| Total Iron Binding Capacity (TIBC) | : 362.99 | µg/dL | 255-450                       |
| Method : Nitroso-PSAP              |          |       |                               |

**Reference:**Beckman kit literature.

Note :- Suggested Clinical Correlation \*

\*\*End Of Report\*\*

Dr.Vijay Babu B,MBBS.MD  
Consultant BiochemistryDr.Sowmya Gayatri .C  
M.B.B.S MD  
Consultant BiochemistryDr.S.A.Nabi Ph.D  
Sr.Biochemist

Processing Location :

Rd #68, Jubilee Hills, Hyderabad- 500033  
Email: centrallab@luciddiagnostics.com

Client : HCL AVITAS PVT LTD

Patient Name : **Mrs.VIDYA SRIKESH**

Age / Gender : **60 Y(s) / Female**

Phone : 9492063115

Ref.Dr. : SELF

Req.No : **DSN102500337** - 2500879502

Report Status : **Final**

Sample Type : Plasma - F

Registered On : 18-10-2025 09:04

Collected On : 18-10-2025 09:07

Received On : 18-10-2025 12:19

Reported On : 18-10-2025 13:33

Scan to Validate



DEPARTMENT OF CLINICAL BIOCHEMISTRY

**Glucose, Fasting (FBS)**

| TEST NAME                     | RESULT | UNITS | BIOLOGICAL REFERENCE INTERVAL                                       |
|-------------------------------|--------|-------|---------------------------------------------------------------------|
| <b>Fasting Plasma Glucose</b> | : 82   | mg/dL | Normal : 70 - 100<br>Prediabetes : 100-125<br>Diabetic : $\geq 126$ |
| <i>Method : Hexokinase</i>    |        |       |                                                                     |

**Interpretation:**

Criteria for diagnosis of Diabetes mellitus.

FPG  $\geq 126$  mg/dL, Fasting is defined as no caloric intake for atleast 8 hours.

**Remarks:**

Fasting hypoglycemia may be observed in persons taking certain diabetes medications, antibiotics and alcoholic beverages. Additionally intestinal disorders, endocrine disorders, insulin surge, nature of diet, reduced food intake than usual and stress are among several other factors. Please correlate clinically.

**Reference:** American Diabetes association guidelines 2021.

Note :- Suggested Clinical Correlation \*

**\*\*End Of Report\*\***

*Vijay*

**Dr.Vijay Babu B,MBBS.MD**  
Consultant Biochemistry

**Dr.Sowmya Gayatri .C**  
**M.B.B.S MD**  
Consultant Biochemistry

**Dr.S.A.Nabi Ph.D**  
Sr.Biochemist



|              |                       |               |                     |
|--------------|-----------------------|---------------|---------------------|
| Patient Name | : Mrs . VIDYA SRIKESH | Reg No        | : DSN102500337      |
| Age/Gender   | : 60 Y(s) / Female    | Reg Date      | : 18-Oct-2025 09:04 |
| Ref.Dr       | : SELF                | Reported Date | : 18-Oct-2025 13:38 |
| Reference    | : HCL AVITAS PVT LTD  |               |                     |

**ULTRASOUND WHOLE ABDOMEN**

**LIVER:** Normal in size (12.2 cms) and echotexture with smooth contours. No focal lesions. Intrahepatic biliary and vascular radicles are normal.

**PORTAL VEIN:** Normal. **CBD:** Normal.

**GALL BLADDER:** Well distended. No evidence of wall thickening.

**Evidence of echogenic sludge with no vascularity and few hyperechoic foci within noted in the gall bladder lumen.**

**SPLEEN:** Normal in size (7.5 cms) and echopattern.

**PANCREAS:** Normal in size and outlines. Parenchymal texture normal. No ductal dilatation. No calcifications / calculi

**AORTA & IVC:** Normal in calibre. No pre / para aortic lymphadenopathy.

**KIDNEYS:** Normal in size, with smooth contours. Parenchymal texture normal. Cortico medullary differentiation well maintained. No calyectases / calculi.

Right Kidney measures 8.8 x 4.1 cms.

Left Kidney measures 9.3 x 4.8 cms.

**URINARY BLADDER:** Well distended. No evidence of wall thickening / calculi.

**UTERUS:** Post menopausal status. Normal in size and anteverted.

Myometrial and endometrial echoes are normal.

Endometrium thickness is normal.

**OVARIES:** Not visualized. Not adequately visualized due to bowel shadows.

No fluid seen in Pouch of Douglas.

No free fluid in the abdomen.

**IMPRESSION:**

**\* Echogenic sludge with few hyperechoic foci within in the gall bladder lumen - Possibility of tumefactive**

**Dr.Sunitha Lingareddy**  
Consultant Radiologist

**Dr.Praveen Jagarlamudi**  
Consultant Radiologist

**Dr.Vikas Reddy P**  
Consultant Radiologist

**Dr.Rajya Lakshmi.D**  
Consultant Radiologist

**Dr.Salila I**  
Consultant Radiologist

  
**Dr.Aditya Abhishek K**  
Consultant Radiologist







Patient Name : **Mrs . VIDYA SRIKESH**  
Age/Gender : **60 Y(s) / Female**  
Ref.Dr : **SELF**  
Reference : **HCL AVITAS PVT LTD**

Reg No : **DSN102500337**  
Reg Date : **18-Oct-2025 09:04**  
Reported Date : **18-Oct-2025 13:38**

**sludge with microlithiasis.**

**SUGGESTED CLINICAL CORRELATION AND FOLLOW UP**

**Dr.Sunitha Lingareddy**  
Consultant Radiologist

**Dr.Praveen Jagarlamudi**  
Consultant Radiologist

**Dr.Vikas Reddy P**  
Consultant Radiologist

**Dr.Rajya Lakshmi.D**  
Consultant Radiologist

**Dr.Salila I**  
Consultant Radiologist

**Dr.Aditya Abhishek K**  
Consultant Radiologist





Patient Name : **Mrs . VIDYA SRIKESH**  
Age/Gender : **60 Y(s) / Female**  
Ref.Dr : **SELF**  
Reference : **HCL AVITAS PVT LTD**

Reg No : **DSN102500337**  
Reg Date : **18-Oct-2025 09:04**  
Reported Date : **18-Oct-2025 14:55**

**X-RAY CHEST PA VIEW**

Soft tissues / bony cage normal.

Trachea and Mediastinal structures are normal.

Heart size and configuration are normal.

Aorta and pulmonary vascularity are normal.

Lung parenchyma and CP angles are clear.

Bilateral hilae and diaphragmatic contours are normal.

**IMPRESSION: NORMAL STUDY.**

**SUGGESTED CLINICAL CORRELATION AND FOLLOW UP**

**Dr.Sunitha Lingareddy**  
Consultant Radiologist

**Dr.Praveen Jagarlamudi**  
Consultant Radiologist

**Dr.Vikas Reddy P**  
Consultant Radiologist

**Dr.Rajya Lakshmi.D**  
Consultant Radiologist

**Dr.Salila I**  
Consultant Radiologist

  
**Dr.Aditya Abhishek K**  
Consultant Radiologist





y. u. vidya

16-10-2025 12:05:27 PM  
HR : 91 bpm  
P : 92 ms  
PR : 122 ms  
QRS : 82 ms  
QT/QTc : 336/414 ms  
P/QRS/T : 56/36/26 °  
RV5/SV1 : 0.711/0.506 mV

## Diagnosis Information:

Sinus rhythm

Possible sequence error: V2,V3 omitted

Technician : SELF

Ref-Phys. :

Report Confirmed by:

**DR. SHABARINATH SAMUDRALA**  
MB (GENERAL MEDICINE) DNB (CARDIOLOGY)  
CONSULTANT INTERVENTIONAL CARDIOLOGIST  
REG No: TSMC/FMR/05208

